

THE MATERNAWELL TRAY

May 2026 UPDATE on PILOTS and CONTINUOUS USE

In maternity wards around the world, postpartum hemorrhage remains one of the most dangerous complications of childbirth. It is also one of the most treatable, if it is recognized early.

Yet recognition is often the challenge. For generations, clinicians have relied on visual estimation to assess blood loss during delivery. Research has repeatedly shown that visual estimates are unreliable, often underestimating blood loss by as much as half. The result is delayed diagnosis, delayed treatment, and preventable maternal deaths.

At Maternova, the incorporation of the MaternaWell Tray into our work began with a simple question: What if measuring blood loss during childbirth were as routine and straightforward as measuring blood pressure?

We found the answer in the form of a reusable device designed to sit beneath the mother immediately after delivery, allowing clinicians to collect and directly measure blood loss in real time. Unlike disposable drapes, which must be replaced after every birth, the tray is durable, easy to clean, and designed for repeated use, an important consideration for facilities where resources and supply chains can be unpredictable. This device had been developed by physicians and midwives through many iterations and prototypes.

Today, the MaternaWell Tray is quietly making its way into maternity wards across the world. Pilots are underway or have already occurred in **11 countries (from Mali to Malawi to Sierra Leone, Liberia and Rwanda)**. And the trays are *in continuous use* in South Africa, Zimbabwe, Sierra Leone, Liberia and Cameroon. The trays in continuous use were either piloted and then moved to continuous use or went directly to continuous use without a pilot because of demand and need.

1. SOUTH AFRICA: PEER REVIEWED ARTICLE ON PILOT AND WIDESPREAD ADOPTION

Clinicians in South Africa have responded with remarkable enthusiasm. One provider summarized their experience simply:

“These trays are the best thing that ever happened to us.”

The first pilot comparing the underbuttocks drape to the MaternaWell Tray took place in South Africa at a midwife obstetric unit in Khayelitsha Cape Town and was published as a peer-reviewed study. In the trial there were 63 participants, of which 33 were assigned to the MaternaWell tray and 30 to the Brass-V drape.

- 90% of Birth attendants indicated a desire to use the MaternaWell tray in future deliveries (n=30)
- 87% of Birth attendants indicated a desire to use the Brass-V drape in future deliveries. (n=26)
- 100% of the patients were also in favour of the future use of MaternaWell tray (n=33)
- 93% of the patients were in favour of the future use of the Brass-V drape (n=28)

This peer-reviewed study also checked the estimations of blood loss and found that the difference in visual versus measured blood loss per device was lower for the MaternaWell Tray.

The MaternaWell Tray is now in widespread use in South Africa across multiple provinces. The South African guidelines support the use of the tray. At a recent training of ob/gyn residents, 60% had heard of or had used the tray in their clinical practice.

2. LIBERIA: PILOT STUDY WITH POSITIVE RESULTS

The initial pilot testing of the MaternaWell Tray took place with a local Liberian organization in one county in Grand Bassa to look at acceptability and feasibility of the tray use for both providers and clients. The study included 14 providers and 115 clients.

The team reported positive results from the pilot from both providers and clients. Key findings from the pilot found that the tray was generally acceptable by both health providers and clients:

- 93% (n=14) of providers found the tray very easy or somewhat easy to use
- 91% of clients (n=115) felt good about the facility starting to use the tray
- 94% of clients (n=115) said the tray made them feel safer
- 84% of clients (n=115) said the tray was comfortable

The team co-created some additional PPH data collection tools with the Ministry of Health and County Health Team which were positively received. The team also calculated cost implications which were shared with the MOH as part of the results to support planning and decision-making.

After training and initial implementation, the trays quickly became part of routine practice across . Providers reported positive experiences from both clinicians and mothers, and local teams have begun advocating for their broader inclusion in national guidelines.

“On-going anecdotal feedback from the facilities suggests that both clients and providers continue to use the trays and find them acceptable and feasible. “The ease of use, reusability, and improved ability to measure blood loss (and thus trigger action for PPH management) continue to be positive aspects, making the tray more acceptable”.

The team has shared the results with the MOH-Family Health and their Liberia Country Director is part of the national level technical working group for PPH. Due to the positive results on acceptability and feasibility, they are open to include the trays as an option in the updated guidelines for early detection/measurement alongside the calibrated drape and the County Health Team is interested in further roll out of the trays at their other facilities.

3. ZIMBABWE PILOT and CONTINUOUS USE

In Zimbabwe, a 2024-2025 pilot conducted by the United Nations Population Fund compared disposable drapes with the reusable tray in facilities at different levels of care. The evaluation ended earlier than expected, not because of problems, but because the positive results favoring the tray were spotted early. The tray proved easy to implement and more affordable for routine use. 600 trays were purchased by UNFPA Zimbabwe and are in continuous use today.

4. COLOMBIA PILOT UNDERWAY

A pilot comparing the MaternaWell Tray to the drape is being undertaken in Colombia. Details on the size of the study are still being determined.

5. ANGOLA PILOT UNDERWAY

In Angola, partners are currently in the training phase of a pilot with plans to begin collecting data in the coming months. The pilot will equip midwives with the MaternaWell Tray (already in the country) and assess the usability and feasibility of the trays in the Angolan context.

6. MALI PILOT UNDERWAY

A pilot in Mali is just beginning, with the trays already in the country.

7. MALAWI PILOT UNDERWAY

Partners in Health has deployed trays to Malawi for a pilot phase which is underway. Initial feedback is positive.

8. LAOS PILOT STARTING

A pilot in Laos is being planned in collaboration with the Ministry of Health, comparing the drape to the MaternaWell Tray. 435 trays are being deployed in this pilot, and compared with 19,000 drapes

9. RWANDA PILOT UNDERWAY

Partners in Health has deployed the trays in Rwanda. The trays are in a pilot phase in 56 clinics and also in Butaro and Rwinkwavue Hospitals.

The Associate Director of the PIH MNCAH program in Rwanda stated the following:

Following the utilization of the trays, we have observed their multifaceted roles. They contribute significantly to infection prevention during delivery by effectively containing fluids, thereby ensuring a cleaner and safer process. Additionally, they reduce costs associated with linen cleaning and provide immediate and accurate quantification of blood loss after birth though contribute for early detection and management of postpartum hemorrhage. Out of the 210 trays received, distribution has been made across 56 health facilities.

10. CAMEROON: CONTINUOUS USE OF THE TRAYS

Across the continent in Cameroon, the trays are now in use at Regional Hospital Limbe and Regional Hospital Buea through the Cameroon Arizona Partnership, where discussions are underway with the Ministry of Health about expanding postpartum hemorrhage prevention programs to additional facilities.

"I think the [MaternaWell] tray is the best thing that has happened in my career." (Nurse Midwife, Cameroon)

11. SIERRA LEONE CONTINUOUS USE OF MATERNAWELL TRAY

In Sierra Leone, there was no pilot for some organizations. Rather, the trays were introduced immediately into clinical practice by two different organizations. The trays are also being piloted by PIH in the country's PIH Maternal Center of Excellence. There, head midwife Isata Dumbuya noted that even in the early days of use, the trays have improved the accuracy of clinical reporting and planning around blood loss.

Dr. Fatu Forna of the Mama Pikin Foundation described how the trays change the moment when a midwife must decide whether a patient is hemorrhaging:

"The trays empower the midwives to call a hemorrhage a hemorrhage with confidence."

Confidence matters. And timing matters. When blood loss is measured objectively, providers can act sooner, mobilize medications faster, and coordinate emergency care more effectively.

12. GUAM, UZBEKISTAN, ALBANIA, UGANDA, BOTSWANA CUSTOMERS

Beyond Africa, interest continues to grow. Hospitals in Guam have begun using the trays at Guam Regional Medical City, while clinicians in Botswana, Uganda, Albania and Uzbekistan are exploring their integration into maternity care with small orders already delivered and in use.

CONCLUSION

What makes the MaternaWell Tray notable is not just its design, but its practicality. In many of the settings where maternal mortality remains highest, hospitals must make difficult choices about which technologies they can realistically maintain. A reusable device that can serve thousands of births offers a different kind of sustainability.

As more facilities begin using the MaternaWell Tray, the most valuable insights will continue to come from the people who use it every day: midwives, nurses, and obstetricians working on the front lines of childbirth.

Their feedback will shape the next chapter of this story of innovation.

References

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